

Policy on Laboratory Services

«Hospital Name»

Document control

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Prepared by	Laboratory In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

Laboratory services are provided as per the scope of services of the organisation — so that results are available round the clock where clinical care needs them, handled safely from requisition to disposal, reported on time, and quality-assured.

1. Purpose

This policy says how «Hospital Name» provides laboratory services commensurate with its clinical scope, keeps infrastructure and human resources adequate, uses qualified staff under pathologist/microbiologist/biochemist supervision, follows written guidance for the specimen journey, delivers results within defined turnaround time (TAT), intimates critical results immediately, with read-back, reports in a standardised manner, recalls or amends reports when needed, and outsources unavailable tests under a quality-assured MoU.

The chapter intent is that laboratory services are provided by competent staff so that patient care is continuous and diagnostic results support safe clinical decisions.

This policy does not cover laboratory quality assurance, laboratory safety, infection control, or biomedical-waste compliance — those are covered in other hospital policies.

2. Scope

This policy applies to the laboratory in-charge, pathologist, microbiologist, biochemist, laboratory technologists, treating doctors who requisition tests, nurses who collect or transport specimens, and the Quality Coordinator at «Hospital Name».

It covers the full scope of laboratory service provision: round-the-clock availability; infrastructure and layout; human resources and qualified supervision; written guidance for the specimen journey; turnaround time (TAT) management; critical-result intimation; standardised reporting; recall and amendment; and outsourcing arrangements with MoU.

Boundaries:

- The laboratory quality assurance programme and the laboratory safety programme (IQC/EQA, MSDS, PPE training) are covered in other hospital policies. This policy owns service scope, specimen process, TAT, critical intimation, reporting and outsourcing.
- HIC and the organisation's biomedical-waste / infection-control documents own waste segregation categories and statutory waste compliance. This policy requires disposal per those documents; it does not import that statute into paragraph 2 of the disclaimer.
- MoU structure for outsourced diagnostic services is covered in other hospital policies; this policy requires an MoU that incorporates quality assurance for laboratory outsourcing.
- Displayed clinical services and registration identity are covered in other hospital policies; the unique identification number used on specimens comes from registration.

3. Policy standards

«Hospital Name» provides laboratory services commensurate with the healthcare services it offers, available round the clock so that patient care is not disrupted, with emergency-management test results available on its premises. Infrastructure, layout, human resources and qualified supervision are adequate for the defined scope. Specimens are requisitioned, collected, identified, handled, transported, processed and disposed of according to written guidance. Results are available within a defined TAT, critical results are intimated immediately with documented read-back, and reports follow a standardised format. There is a mechanism to recall or amend reports. Tests not available in-house are outsourced only to organisations with a quality assurance system under a written MoU.

«Hospital Name» does not delay a critical laboratory result, and does not release an altered outsourced report as if it were an in-house result.

4. Non-negotiable rules

1. Do not offer a laboratory service outside the approved test menu without laboratory in-charge agreement and menu update or a documented outsourced arrangement.
2. Do not accept or process a specimen that is unlabelled, mislabelled, or collected without following written guidance.
3. Do not report a result without the authorised signatory required by statute and by this hospital's authorisation list.
4. Do not leave a critical result uncommunicated once the report is ready.
5. Do not alter or modify content of an outsourced laboratory report.

6. Do not outsource a test to a laboratory that has no documented quality assurance system and no MoU with this hospital.
7. Do not leave a recalled report in clinical areas, the medical record, LIS or HIS after recall without replacement by the amended report.
8. Staff who see a laboratory service rule broken report it the same shift to the Laboratory In-Charge or the Medical Superintendent.

5. What we do

5.1 Scope of laboratory services

Scope of the laboratory services at «Hospital Name» is commensurate with the healthcare services the organisation provides. The laboratory in-charge maintains a test menu listing every test available in-house and every test outsourced. The menu is reviewed at least annually (the organisation's adopted cycle) and whenever clinical services change.

Laboratory services are available round the clock so that patient care is not disrupted. Test results required for emergency management are available within the premises. Tests not on the menu are not offered as in-house services.

5.2 Infrastructure and layout

The infrastructure — physical space and equipment — is adequate to provide the defined scope of services. Reports must not be delayed for lack of adequate equipment.

The layout of the laboratory prevents cross-contamination. Clean and dirty workflows are separated; specimen receipt, processing and reporting areas are arranged so that contamination routes are interrupted. The laboratory in-charge reviews layout fitness at least annually (the organisation's adopted cycle) and after any renovation or major equipment change.

5.3 Human resources

Human resource is adequate to provide the defined scope of services. The number of laboratory personnel is commensurate with workload, with sufficient staff for each shift and for emergencies. Reports must not be delayed for lack of adequate human resource, including personnel authorised to report results.

5.4 Qualified performance and supervision

Qualified and trained personnel perform and supervise the investigations and report the results. Staff employed in the laboratory hold an appropriate degree and are trained to carry out the tests they perform.

A Pathologist, Microbiologist and Biochemist supervise the staff for their respective disciplines as this hospital's scope requires. Statutory requirements regarding the authorised signatory are adhered to. The laboratory in-charge keeps the current list of authorised signatories and does not release reports outside that list.

5.5 Written guidance for requisition through disposal

Requisition for tests, collection, identification, handling, safe transportation, processing and disposal of a specimen is performed according to written guidance.

The unique identification number from registration is used for identification of the patient. Two-patient identifiers are checked at collection and at receipt. Rejected specimens (haemolysed, clotted, insufficient, mislabelled) are logged with reason and a fresh specimen is requested.

Disposal of waste follows the statutory biomedical-waste duties owned by this hospital's infection-control / waste documents. This step requires laboratory staff to segregate and hand over laboratory waste under those documents.

5.6 Results within defined turnaround time

Laboratory results are available within a defined time frame. The organisation defines the turnaround time for all tests and ensures adequate staff, materials and equipment so that results are available within that frame.

The turnaround time may differ for different tests and is decided based on the nature of the test, criticality and urgency desired by the treating doctor. The laboratory in-charge documents the defined TAT for each test or test category in the laboratory test menu or a separate TAT schedule.

TAT compliance is monitored continuously, based on the College of American Pathologists (CAP) Q-TRACKS quality monitoring programme, which established continuous monitoring of stat and routine outlier turnaround times as recognised laboratory best practice — this is the organisation's adopted practice; NABH does not mandate a specific monitoring frequency here.

5.7 Critical results intimated immediately

Critical results are intimated to the person concerned at the earliest.

Critical limits for tests that require immediate attention for patient management are documented. Critical results of outsourced investigations are included. Critical test results are communicated to a person from the treating team as soon as the test result or report is ready.

The intimation includes: name of the patient; unique ID; date and time of sample collection; test name, result, measure unit and reference range; identity of who communicated the value; identity of the recipient; read-back; and date and time of acknowledgement.

5.8 Standardised reporting

Results are reported in a standardised manner. At a minimum the report includes: the name of the organisation (or, for an outsourced laboratory, the name of that laboratory); the

patient's name; the unique identification number; the reference range of the test where applicable; and the name and signature of the person reporting the test result.

The organisation does not alter or modify anything in the outsourced report. If on this organisation's letterhead, the report includes at least the name of the outsourced laboratory, the date and the reference number of the report given by the outsourced laboratory.

5.9 Recall and amendment of reports

There is a mechanism to address the recall or amendment of reports whenever applicable. When a particular report is recalled, it is withdrawn from clinical areas, medical records, the Laboratory Information System (LIS) and the Hospital Information System (HIS). If already issued to the patient, the amended report is made available with a caution to ignore the earlier one.

5.10 Outsourced laboratory tests

Laboratory tests not available in the organisation are outsourced to organisation(s) based on their quality assurance system.

Written guidance for outsourcing includes: a list of tests for outsourcing; identity of personnel in the outsourced facilities; manner of packaging and labelling of specimens; a methodology to check the performance of the service. The organisation has a Memorandum of Understanding (MoU) / agreement that incorporates quality assurance and the requirements of this standard. The panel and MoUs are reviewed at least annually (the organisation's adopted cycle).

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that laboratory services match clinical scope, remain available round the clock for defined emergency tests, and meet TAT and critical-result duties.
Laboratory In-Charge	Maintains the test menu, staffing plan, authorised-signatory list, written specimen guidance, TAT definitions, critical-limit list, standardised report format, recall mechanism and outsourced panel with MoUs.
Pathologist / Microbiologist / Biochemist	Supervise investigations and reporting in their disciplines; act as or nominate authorised signatories as statute and hospital authorisation require.
Laboratory technologists	Perform tests they are trained for; follow specimen guidance; communicate critical results when designated; do not alter

	outsourced reports.
Treating doctors and ward nurses	Requisition appropriately; receive critical-result intimation with read-back; act on amended reports after recall.
Quality Coordinator	Audits this policy; tracks CAPA when TAT, critical-result or recall defects recur.

7. Quality monitoring (RCA → CAPA)

What is monitored:

- Test menu current and commensurate with clinical services; emergency tests available on premises.
- Infrastructure, layout and staffing adequate for scope.
- Authorised-signatory list current; unqualified reporting absent.
- Specimen guidance followed; rejection log reviewed.
- TAT compliance by category.
- Critical-result communication immediately, with full documentation and read-back.
- Standardised report format; outsourced letterhead rules followed; no alteration of outsourced content.
- Recall/amendment log complete with withdrawal evidence and CAPA.
- Outsourced panel and MoUs current and quality-based.

Root-cause analysis is required when the same laboratory service defect recurs; the organisation's defined recurrence threshold applies, as documented in the hospital's Root Cause Analysis procedure.

This policy is reviewed annually, and sooner when clinical scope, equipment or outsourcing arrangements change.

8. Training and staff acknowledgement

All laboratory staff, nurses who collect or transport specimens and doctors who requisition tests are trained on this policy at induction and once a year after that. Training covers specimen guidance, TAT, critical-result intimation with read-back, standardised reporting, recall/amendment and outsourcing rules.

Staff acknowledgement

I have read the Policy on Laboratory Services of «Hospital Name». I will follow the processes described.

Name: _____ Designation: _____

Department / floor: _____ Date: _____

Signature: _____

(One row per staff member. The Quality Coordinator holds signed acknowledgements with the induction record.)

9. Distribution

Official master copy: office of the Medical Superintendent, «Hospital Name», with the Quality Coordinator.

Copies issued to: laboratory; emergency; every in-patient ward; out-patient; nursing administration; intensive-care areas if present.

The current version is available to all staff at the front-office policy file and, if the hospital keeps an intranet, at staff intranet / policies.

When a new version is issued, take old copies out of use.

10. Abbreviations

Abbreviation	Meaning
AAC	Access, Assessment and Continuity of Care (NABH HCO chapter)
CAPA	corrective and preventive action
HCO	Hospital (Full Accreditation programme)
HIS	Hospital Information System
HIC	Hospital Infection Control
LIS	Laboratory Information System
MoU	Memorandum of Understanding
NABL	National Accreditation Board for Testing and Calibration Laboratories
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	objective element
PPE	personal protective equipment
PSQ	Patient Safety and Quality
ROM	Responsibilities of Management
TAT	turnaround time

11. Traceability to NABH HCO Full Accreditation 6th Edition AAC.6

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
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AAC.6. a	Commitment	Scope of the laboratory services is commensurate to the services provided by the organisation.	Section 3; 5.1	Laboratory In-Charge (maintain menu); Medical Superintendent (approve scope)
AAC.6. b	Commitment	The infrastructure (physical and equipment) is adequate to provide the defined scope of services; layout prevents cross-contamination.	Section 3; 5.2	Laboratory In-Charge (manage); Medical Superintendent (resource)
AAC.6. c	Commitment	Human resource is adequate to provide the defined scope of services.	Section 3; 5.3	Laboratory In-Charge (roster); Medical Superintendent (resource)
AAC.6. d	Commitment	Qualified and trained personnel perform and supervise the investigations and report the results.	Section 3; 5.4	Pathologist/Microbiologist/Biochemist (supervise); Laboratory In-Charge (authorised signatories)
AAC.6. e	Commitment *	Requisition for tests, collection, identification, handling, safe transportation, processing and disposal of a specimen is performed according to written guidance.	Section 3; 5.5	Laboratory In-Charge (own SOPs); technologists and collecting nurses (follow)

AAC.6.f	Commitment *	Laboratory results are available within a defined time frame.	Section 3; 5.6	Laboratory In-Charge (define and monitor TAT); Quality Coordinator (audit)
AAC.6.g	Commitment *	Critical results are intimated to the person concerned at the earliest.	Section 3; 5.7	Designated laboratory personnel (intimate); treating team (read-back); Laboratory In-Charge (limits list)
AAC.6.h	Commitment	Results are reported in a standardised manner.	Section 3; 5.8	Laboratory In-Charge (format); authorised signatory (sign); technologists (do not alter outsourced content)
AAC.6.i	Commitment *	There is a mechanism to address the recall / amendment of reports whenever applicable.	Section 3; 5.9	Laboratory In-Charge (run mechanism); Quality Coordinator (CAPA)
AAC.6.j	Commitment *	Laboratory tests not available in the organisation are outsourced to organisation(s) based on their quality assurance system.	Section 3; 5.10	Laboratory In-Charge (panel and MoU); Medical Superintendent (approve MoU)

12. Required Records / Evidence Checklist

Records the hospital holds under this policy, listed by objective element.

AAC.6.a — Scope commensurate with services

- Laboratory test menu listing in-house and outsourced tests.
- Annual review of menu against clinical services.
- Record that emergency-management tests required by scope are available on premises.

AAC.6.b — Infrastructure and layout adequate

- Equipment inventory with model, serial and maintenance schedule link.
- Layout / workflow description showing separation that prevents cross-contamination.

- Annual infrastructure adequacy review.

AAC.6.c — Human resource adequate

- Staff list by shift including emergency cover.
- Annual staff-to-workload review.
- List of personnel authorised to report results.

AAC.6.d — Qualified performance and supervision

- Qualification and training records for laboratory staff.
- Supervision arrangement by Pathologist, Microbiologist and Biochemist as applicable.
- Current authorised-signatory list meeting statutory requirements.

AAC.6.e — Written guidance for requisition through disposal

- Written guidance covering requisition, collection, identification, handling, transport, processing and disposal.
- Specimen rejection log with reasons.
- Evidence of unique ID (and lab number if used) on specimens and requisitions.

AAC.6.f — Results within defined TAT

- Defined TAT for all tests or test categories.
- Continuous TAT compliance monitoring records (per CAP Q-TRACKS programme adopted by the organisation).
- Escalation records for persistent TAT breaches.

AAC.6.g — Critical results intimated immediately

- Documented critical limits and biological reference intervals (or evaluated published intervals).
- Critical-result communication log with patient ID, result, caller, recipient, read-back, date and time.
- List of personnel authorised to report critical results.

AAC.6.h — Standardised reporting

- Standardised report template with minimum required fields.
- Sample outsourced reports on outsourced letterhead or organisation letterhead with required attribution.
- Audit finding that outsourced report content was not altered.

AAC.6.i — Recall / amendment mechanism

- Recall/amendment log with reason and date.
- Evidence of withdrawal from clinical areas, medical records, LIS and HIS.
- Amended report issued to patient with caution where previously issued; CAPA record.

AAC.6.j — Outsourced laboratory tests

- Written outsourcing guidance with test list, transport contacts, packaging and performance checks.
- Current MoU/agreement incorporating quality assurance.
- Annual review of outsourced laboratory performance.

13. References

- National Accreditation Board for Hospitals and Healthcare Providers (NABH), Guidebook to Accreditation Standards for Hospitals, 6th Edition — Access, Assessment and Continuity of Care (AAC), standard AAC.6.
- ISO 15189:2022 and NABL 112 — guidance on medical laboratory quality and competence.
- College of American Pathologists, Q-TRACKS Quality Indicator Monitoring Programme — Novis DA, et al. “Continuous monitoring of stat and routine outlier turnaround times: two College of American Pathologists Q-TRACKS monitors in 291 hospitals.” Arch Pathol Lab Med.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. This policy does not import the Consumer Protection Act, 2019, the Clinical Establishments Act, 2010, or the Mental Healthcare Act, 2017 as a checklist. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

«Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed. The clinical and technical content reflects recognised national and international guidance current at the date of preparation.

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